

Sample Surgical Center

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DISCHARGE SUMMARY

Patient: Roe, Thomas C.	DOB: 11/22/1971	MRN: SSC-2025-003217
Admission Date: 02/03/2025	Discharge Date: 02/05/2025	Attending: Dr. John Lee, MD
Service: Orthopedic Surgery	Room: 3-South, Bed 308A	Insurance: Sample Insurance Company

CHIEF COMPLAINT

Right knee pain, mechanical instability, and progressive functional limitation.

HISTORY OF PRESENT ILLNESS

Mr. Roe is a 53-year-old male with a 4-year history of progressive right knee osteoarthritis who presents for elective total right knee arthroplasty. Conservative management including physical therapy, NSAIDs, corticosteroid injections (last injection 09/2024), and hyaluronic acid supplementation have failed to provide sustained relief. Pre-operative MRI demonstrated bone-on-bone changes in the medial compartment with Grade IV chondromalacia. Patient is a non-smoker, BMI 28.2, and was medically cleared by his PCP and cardiologist prior to surgery.

OPERATIVE PROCEDURE

Right total knee arthroplasty (CPT 27447) performed on 02/03/2025 under spinal anesthesia with sedation. Zimmer NexGen CR implant system utilized. Tourniquet time: 62 minutes. EBL: 180 mL. No intraoperative complications. Patellar resurfacing performed. All components well-fixed with excellent alignment confirmed on intraoperative fluoroscopy.

HOSPITAL COURSE

Post-operative course was unremarkable. Patient achieved 0-90 degrees ROM on POD 1 with physical therapy. Pain well controlled with multimodal regimen. Hemoglobin stable at 11.2 g/dL (pre-op 14.1). No signs of DVT or infection. Surgical site clean, dry, and intact with staples in place. Patient ambulating with rolling walker 25 feet independently by discharge.

DISCHARGE DIAGNOSES

ICD-10	Description
M17.11	Primary osteoarthritis, right knee
Z96.651	Presence of right artificial knee joint
Z87.39	History of musculoskeletal disorders

MEDICATIONS AT DISCHARGE

Medication	Dose	Frequency	Duration
Oxycodone/Acetaminophen 5/325	1-2 tabs	Q4-6H PRN	2 weeks
Celecoxib	200 mg	Daily	4 weeks

Aspirin	325 mg	BID	4 weeks (DVT prophylaxis)
Docusate Sodium	100 mg	BID	While on opioids
Cefazolin 2g IV → Cephalexin	500 mg	QID	5 days post-op

FOLLOW-UP INSTRUCTIONS

- 1. Orthopedic follow-up with Dr. Lee in 2 weeks for staple removal (appt 02/19/2025)
- 2. Home physical therapy 3x/week starting 02/06/2025 (order placed)
- 3. Knee immobilizer at night for 2 weeks
- 4. Ice 20 min every 2 hours while awake for first 72 hours
- 5. No driving until cleared by surgeon
- 6. Return to ED if: fever >101.5°F, increasing redness/drainage at incision, calf pain/swelling, chest pain, or shortness of breath

John Lee, MD, FAAOS
Department of Orthopedic Surgery
Electronically signed 02/05/2025 16:08 MST